



PROCEDURE

Resuscitation and Responding to Clinical Deterioration MET Review and Code Blue

Scope (Staff):	All staff
Scope (Area):	Perth Children's Hospital (PCH)

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

If you have any feedback or comments to improve our clinical policies or our systems, please tell us by filling in the [feedback form](#)

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Aim

To outline the medical emergency processes at Perth Children's Hospital (PCH) for Medical Emergency Team (MET) or Code Blue team in responding to an acute deterioration in an inpatient's clinical status. This includes the response to a medical emergency for a staff member or visitor.

Risk

Delay or failure to recognise clinical deterioration or a delay in the commencement of effective cardiopulmonary resuscitation procedures can lead to a life-threatening event with subsequent mortality or significant morbidity.

Background

Cardiopulmonary arrest in children is an uncommon event. It most commonly occurs as a result of hypoxia and respiratory failure.¹ Circulatory failure and central neurological failure may also lead to cardiorespiratory arrest.¹ Abnormalities in vital signs such as blood pressure, consciousness, respiratory rate, heart rate, and oxygen saturation are common prior to the occurrence of these serious adverse events.² The earlier the clinical deterioration is detected and treated the better the outcome is likely to be.²⁻⁵ In addition to rapid response teams, CAHS PCH utilise an ESCALATION system for recognising and responding to acute deterioration that incorporates a track and trigger system.

Definitions

Clinical handover: The transfer of professional responsibility and accountability for some or all aspects of a patient's or a group of patients' care to another person or professional group on a temporary or permanent basis.

Digital medical record: this is the digitized version of the patient medical record, abbreviated to DMR.

Patient medical record: Documented account of a person's health, illness or treatment in hard copy or electronic format including bedside documentation, integrated progress notes, treatment orders and investigations.

CNC: Clinical Nurse Consultant

CNS: Clinical Nurse Specialist

MET: Medical Emergency Team

PCC: Paediatric Critical Care

NICU: Neonatal Intensive Care Unit

STARS: Safety Team After-hours Response Service

Key Points

- Staff will assess and recognise patients who are at risk of clinical deterioration or whose clinical condition is deteriorating and will respond to their needs in an appropriate and timely manner.^{1, 2, 3}
- Patients with tracheostomies, or those requiring long term ventilation, may have patient specific resuscitation information on their airway profile. The airway and ventilation profiles are available in the patients Digital Medical Record (DMR). *Do not store in the patient's file.* Recognition and response systems must apply at all times for all patients and in all locations of the hospital^{2, 4}. It is mandatory to follow the Early Warning Score Escalation Pathway and escalate care when there is a trigger as per the Paediatric Acute Recognition and Response Observation Tool.
- Any variation to the escalation pathway must be documented in the modification section of the Recognition and Response Tool with a documented plan in the patient's unit medical record (UMR) or e-progress note in the digital medical record (DMR).
- Staff will utilise the age appropriate Recognition and Response Tool to enable recognition of clinical deterioration and respond to patient needs in an appropriate and timely manner.¹⁻³
- Patients will be systematically assessed by clinicians at intervals appropriate to their clinical status.
- All clinical observations will be documented on the Recognition and Response Tool- unacceptable parameters must be recognised and interpreted and care escalated in accordance with the Escalation Pathway or at any time the clinician and/or family has concerns for a patient's clinical condition.
- Patients of concern will be identified and highlighted at handover and on in. Clinical Manager (iCM).
- The circumstances leading to a patient's clinical deterioration and the escalation response will be clearly communicated to the responding clinician or emergency response team (MET or Code Blue) using iSoBAR format in accordance with CAHS Policy [Communicating for Safety](#) and PCH Procedure: [Clinical Handover-Multidisciplinary](#).
- At the discretion of the resuscitation team leader, family members may be given the option to be present during resuscitation. Whenever possible, a staff member will be allocated to the parents / carers for support if they choose to remain present.
- All members of the workforce will attend life support training in accordance with their scope of practice and responsibilities. Refer to [Resuscitation Training: Hospital Paediatric Life Support](#) in the Clinical Practice Manual.
- Resuscitation equipment must be maintained and be functional at all times in accordance with [Resuscitation Equipment and Trolley Checking](#) (Clinical Practice Manual)
- Automatic External Defibrillators (AED) are installed in non-clinical areas.

Clinical Staff

- The Advanced Paediatric Life Support (APLS)³ algorithms will be followed for all children requiring resuscitation at PCH (see [Appendix 1](#)).
- Medical and nursing staff must be familiar with the resuscitation equipment and drugs which are commonly used in emergency resuscitation procedures in accordance with the ARC and APLS.^{4, 7}

Non-Clinical Staff

- The Australian Resuscitation Council (ARC) Basic Life Support (BLS) algorithm will be followed by all non-clinical staff who are first responders in a resuscitation event (see [Appendix 2](#)) and by clinical staff working in the community setting e.g., HITH, Ambulatory Care Services. This algorithm is used for resuscitation of adults at PCH.⁸

Documentation

- Refer to the [Clinical Documentation](#) policy.
- The digital medical record e-progress notes enable episodes of clinical deterioration to be identified promptly in the DMR:
 - Use of the “Progress Note-Clinical Deterioration” eForm with the appropriate drop-down box and heading used in the body of the note, when documenting a response to Clinical Deterioration.
 - Progress Note- Specialist Nurse/ Midwife eForm for use by specialist Senior Nurses

The flattened tree view must be used in the DMR to visualise Progress Note headings.

Neonatal Resuscitation

- Staff working in a casual or relieving capacity in the neonatal unit must be familiar with the [Neonatal Resuscitation guidelines](#) and procedures as the CPR ratios will differ.

Guide regarding Age Definitions ^{3, 4}

Infant*	0 to 12 months
Young Child	1 to 8 years
Older Child	9 to 14 years
Adult**	>14 years

* ALL infants including newborns, cared for outside of the neonatal unit, are to be managed as per the infant resuscitation algorithm.⁴

** In accordance with ARC Guideline 12.1; to maintain consistency in teaching and learning at PCH, any patient >14 years will be treated as an adult. If unsure of the patient's age, if they look like a child, then treat as a child.⁴

Medical Emergency Team (MET) Review

- The Medical Emergency Team (MET) operates within the hospital and includes:
 - Paediatric Critical Care (PCC) Registrar or Consultant
 - Afterhours CNS
 - Primary clinical team registrar (in hours)
 - Members of STARS ([Safety Team After-hours Response Service](#); after hours)
 - CNC Resuscitation.
- A request for a MET Review does not activate a **Code Blue** response.
- The MET system is for inpatients only. Urgent care for non-inpatient children and all adults should be through the **Code Blue** system.
- Activation of a MET review will initiate an urgent medical review of the patient when:
 - The early warning score is triggered (8 or more)
 - Staff and / or parent are concerned that the patient's clinical condition is deteriorating
 - Failure to review may lead to a medical emergency (refer to the Observation and Response Chart for escalation of care).
- Primary Consultant/team will be notified by ward staff when a MET Review is activated

Ward Staff Activated MET Review

- A MET Review is activated via a secure text message using the Vocera Web Console or Vocera Smart Phone Application. Provide the following information:
 - Ward location
 - Patient name
 - Reason for MET Review
 - MET initiator's name.
 - Phone extension
- The PCC Registrar will **respond** to the message within 5 minutes, obtain details and coordinate the MET review (and /or other appropriate clinician (e.g., Duty Anaesthetist).
- The Afterhours CNS will attend the MET call with the PCC Registrar when on shift (1500 – 2230 Mon-Fri and 0930 – 2230 Sat and Sun).

Resuscitation and Responding to Clinical Deterioration: MET Review and Code Blue

- If the PCC Registrar does not respond to the message, the MET review will automatically escalate to the PCC Shift Coordinator or the afterhours CNS if on shift (see hours above)
- If there is no response within a further 5 minutes, a **Code Blue** will be activated by the staff member who requested the MET review.
- Ward staff will contact the primary team (or relevant registrar after hours) to inform that a MET review has been activated. The primary team / registrar should attend the ward immediately.
- Staff will complete the MET / **Code Blue** Event Record MR 873. The original will be retained in the Unit Medical Record Number (UMRN) notes and a copy sent to the Chair of the Resuscitation Committee.

Family Activated MET Review

- Parents / Carers may activate a MET Review as per the 'CARE call' escalation pathway (Refer to the [Aishwarya's CARE Call: Family Escalation](#))
 - **Care call** guides families on how to escalate care via the recognition and response system when they are concerned about their child's condition.
- Families can access CARE Call information on the back of the admission packs, PES information system and other electronic safety screens. Flyers can be provided to families on inpatient areas. "Are you worried" flyers and posters also available.
- **Note:** If the Telecommunications Centre is contacted by a parent or clinical staff member to initiate a MET review, the call will be diverted to activate a **Code Blue**.

If there is any doubt as to whether to request a MET Review or call **CODE BLUE, a **CODE BLUE** must be activated**

A Code Blue (55) provides immediate medical attention or advanced life support via the Code Blue Emergency Team

- A **Code Blue** is activated by dialling the Telecommunications Centre on extension '**55**' and stating:
 - Hospital: 'PCH'
 - Code: 'Code Blue'
 - Location: 'level, department / area'
 - Caller's name and designation.
 - This may be completed using a range of communication devices (e.g., landlines, Vocera) depending on availability at time of emergency event.

Resuscitation and Responding to Clinical Deterioration: MET Review and Code Blue

- The **Code Blue response** provides immediate medical attention or advanced life support intervention by a team with advanced resuscitation skills.
- The Code Blue team will attend to medical emergencies for all staff, patients and visitors within the physical boundaries of PCH (see [Appendix 3](#)).
 - Responders to emergencies outside the boundary will be required to phone '000'.
- Adults who are attended by the **Code Blue** team at PCH, and require ongoing care, will, once stabilised, be transferred to the Sir Charles Gairdner Hospital (SCGH) Emergency Department (ED),
 - The SCGH ED Duty Consultant must be notified prior to transferring the patient via the Link Bridge and lift to SCGH ED so that the most appropriate entry point to the ED can be planned.
- Refer to [Emergency Procedures Manual: Code Blue](#) for more details and Action Cards for a Code Blue.

Code Blue Team Members

In Hours	After Hours
PCC Registrar Emergency Department Registrar PCC Consultant on duty PCC Shift Coordinator (a second PCC nurse will attend when possible) PCC Clinical Nurse Manager Neonatal Registrar Admitting Medical Registrar Hospital Clinical Manager Support Services Staff Member CNC Resuscitation	PCC Registrar Emergency Department Registrar Anaesthetic Registrar STARS team members Neonatal Registrar Afterhours CNS PCC Shift Coordinator (a second PCC nurse will attend whenever possible) PCC Consultant on call Hospital Clinical Manager Support Services Staff Member Security

- Primary Consultant/team will be notified by ward staff when a **Code Blue** is activated
- The **Code Blue** team is responsible for the delivery of care and delegation of roles to achieve: ²
 - Advanced airway management
 - Vascular access

- Administration of resuscitation drugs
- Advanced resuscitation skills
- Defibrillation or cardioversion
- Post resuscitation care.
- Refer to [Appendix 4](#) for Responder Roles in Resuscitation.

Hospital in the Home (HiTH) – Escalation of Care

- Escalation of care will be initiated according to the HiTH Action Plan for Escalation of Care or Sepsis Recognition Escalation Pathway. In response to a medical emergency, HiTH staff must:
 - Initiate a medical emergency 000 call ask for urgent ambulance assistance
 - Assess the patient and initiate appropriate clinical care e.g., commence basic life support
 - Provide an iSoBAR clinical handover to the paramedics when they arrive and complete documentation in the patients' medical record or DMR.
 - The HiTH Nurse (or Hospital Clinical Manager after 2200 hours) will notify the:
 - Emergency Department (ED) Consultant
 - ED Nurse Shift Coordinator
 - Treating medical team
 - Hospital Clinical Manager (HCM).
- The HiTH Nurse (or HCM after 2200 hours) will arrange for the patient's medical record to be available for the review.
- Once the child has been assessed and treated in ED, the HiTH Nurse will be informed by the Emergency Department's Coordinator as to whether the child is to:
 - Remain on HiTH - the management and care may need reassessment by the patients treating team
 - Be admitted to a hospital inpatient ward - the HiTH Nurse or afterhours HCM will ensure the transfer is reflected in WebPAS.
- **N.B.** The cost of ambulance transfer is covered for all HiTH patients even if family are not covered with private health
- Escalation of care for medical emergencies occurring for patients in the care of HiTH will be initiated according to the HiTH escalation pathways located on the **Home and Community Visits Risk Assessment Form** *[under development]*.
- Refer to [Appendix 5: Hospital in the Home \(HiTH\) Early Warning Score Escalation Pathway](#) and [Appendix 6: HiTH Paediatric Sepsis Recognition Escalation Pathway](#)

Related CAHS internal policies, procedures and guidelines
Adrenaline (epinephrine) Monograph (Pharmacy Manual)
Clinical Documentation – (CAHS Policy Manual)
Clinical Handover – PCH Multidisciplinary (PCH Operational Manual)
Mandatory and Core Requirement Training (CAHS Policy)
Observation, Monitoring and Children's Early Warning Tool (Clinical Practice Manual)
Resuscitation Equipment and Trolley Checking (Clinical Practice Manual)
STARS – Safety Team After-hours Response Service (PCH Policy Manual)

References and related external legislation, policies, and guidelines
1. Advance Life Support Group (ALSG). Advanced Paediatric Life Support: A Practical Approach to Emergencies, 6th Edition. Wiley. England 2017
2. Australian Commission on Safety and Quality in Health Care; Recognising and Responding to Acute Deterioration Standard: Background to this standard, 2019. Recognising and Responding to Acute Deterioration Standard Australian Commission on Safety and Quality in Health Care
3. Australian Commission on Safety and Quality in Health Care; National Standard 8 Recognising and Responding to Acute Deterioration. Clinical governance and Quality Improvement to support recognition and response systems, 2019. Clinical governance and quality improvement to support recognition and response systems Australian Commission on Safety and Quality in Health Care
4. Australian Resuscitation Council. Section 12.1 – introduction to Paediatric Advanced Life Support. August 2018. Available from: http://resus.org.au/guidelines/ [Accessed: 1 st June 2021]
5. Australian Resuscitation Council ARC. Guideline 12.6. Techniques in Paediatric Advanced Life Support. January 2016. Available from: http://resus.org.au/guidelines/ [Accessed: 1 st June 2021]
6. Australian Resuscitation Council Guideline 12.2 Advanced Life Support for Infants and Children Diagnoses and Management. March 2016 Available from: http://resus.org.au/guidelines/ [Accessed: 1 st June 2021]

References and related external legislation, policies, and guidelines
7. Australian Resuscitation Council. Guideline 8 – Cardiopulmonary Resuscitation. April 2021. Available from: http://resus.org.au/guidelines/ [Accessed: 1 st June 2021]
8. Australian and New Zealand Resuscitation Council. Standards for Resuscitation: Clinical Practice and Education. June 2014. Available from: https://resus.org.au/standards-for-resuscitation-clinical-practice-and-education/
9. Mayo Healthcare Pty Ltd. Disposable Manual Resuscitator. Information for Use. Australia n.d.
10. Laerdal Silicone Resuscitator Available from: https://laerdal.com/au/products/medical-devices/airway-management/laerdal-silicone-resuscitator/ [Accessed: 1 st June 2021]
11. Bonafide, C. P., Localio, A. R., Roberts, K. E., Nadkarni, V. M., Weirich, C. M., & Keren, R. (2014). Impact of rapid response system implementation on critical deterioration events in children. JAMA Pediatrics, 168(1), 25-33.

Useful resources (including related forms)
Australian Resuscitation Council
CARE call
Emergency Procedures Manual : PCH Code Blue Emergency Procedures
Hospital Paediatric Life Support (Annual competency via iLearn)
Recognising and Responding to Acute Deterioration Policy (WA Mandatory Policy 0086/18)
Starship Children's Health Clinical Guideline: Cardiopulmonary Resuscitation
Take 5 MET Review and Code Blue
Vocera Quick Reference Guide – How to request a MET review (displayed in clinical areas)

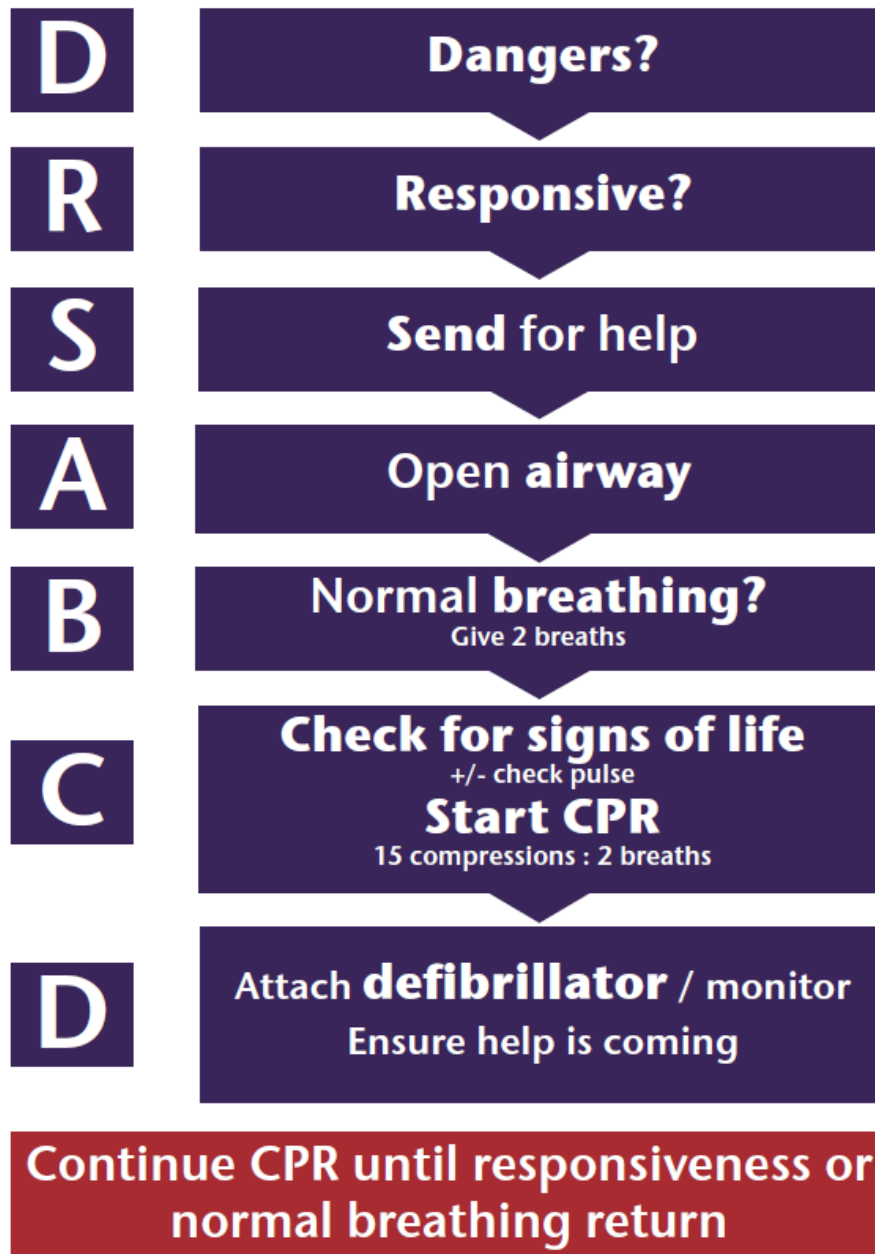
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 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

Appendix 1: Paediatric Algorithm in Hospital

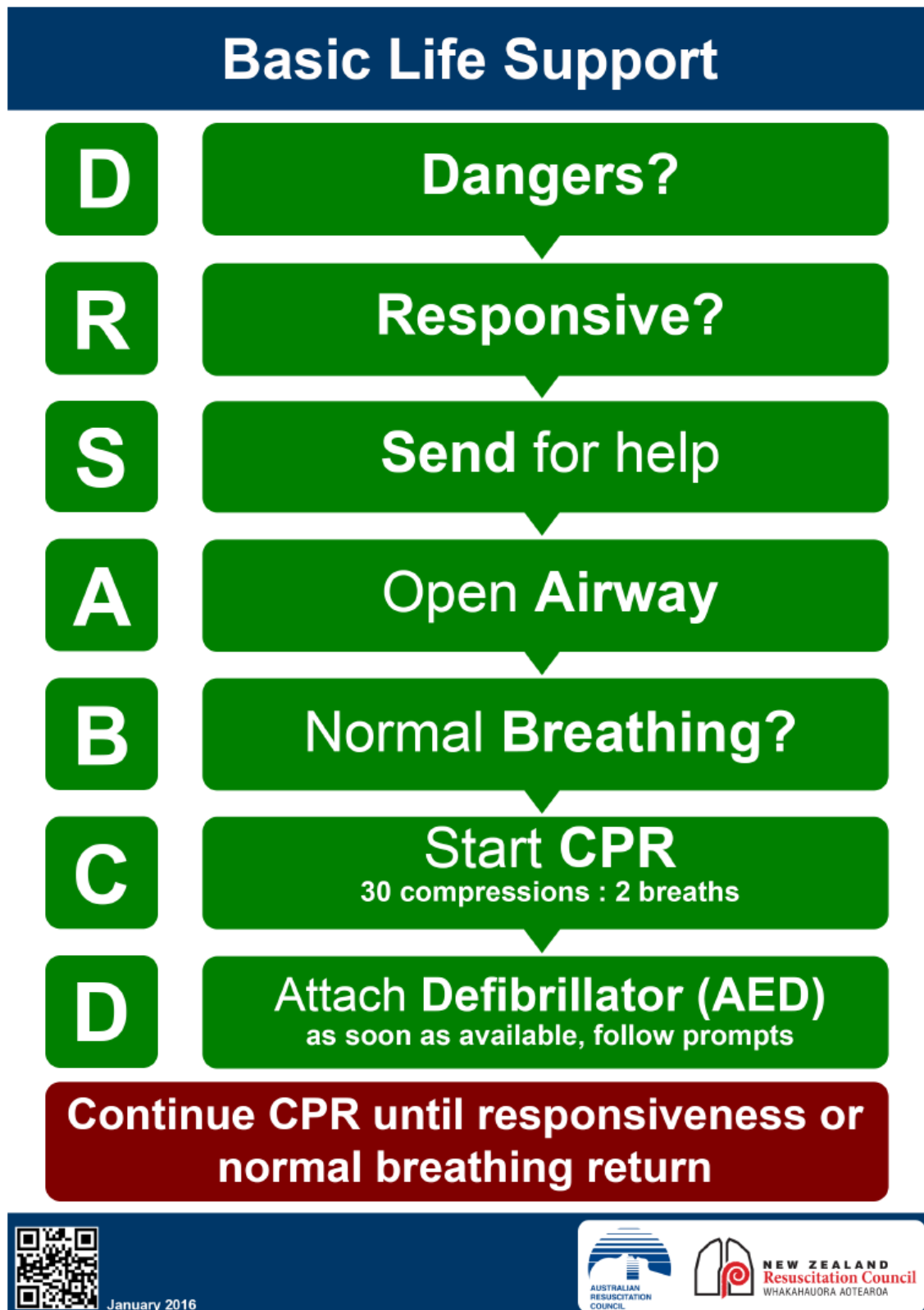


Paediatric basic life support



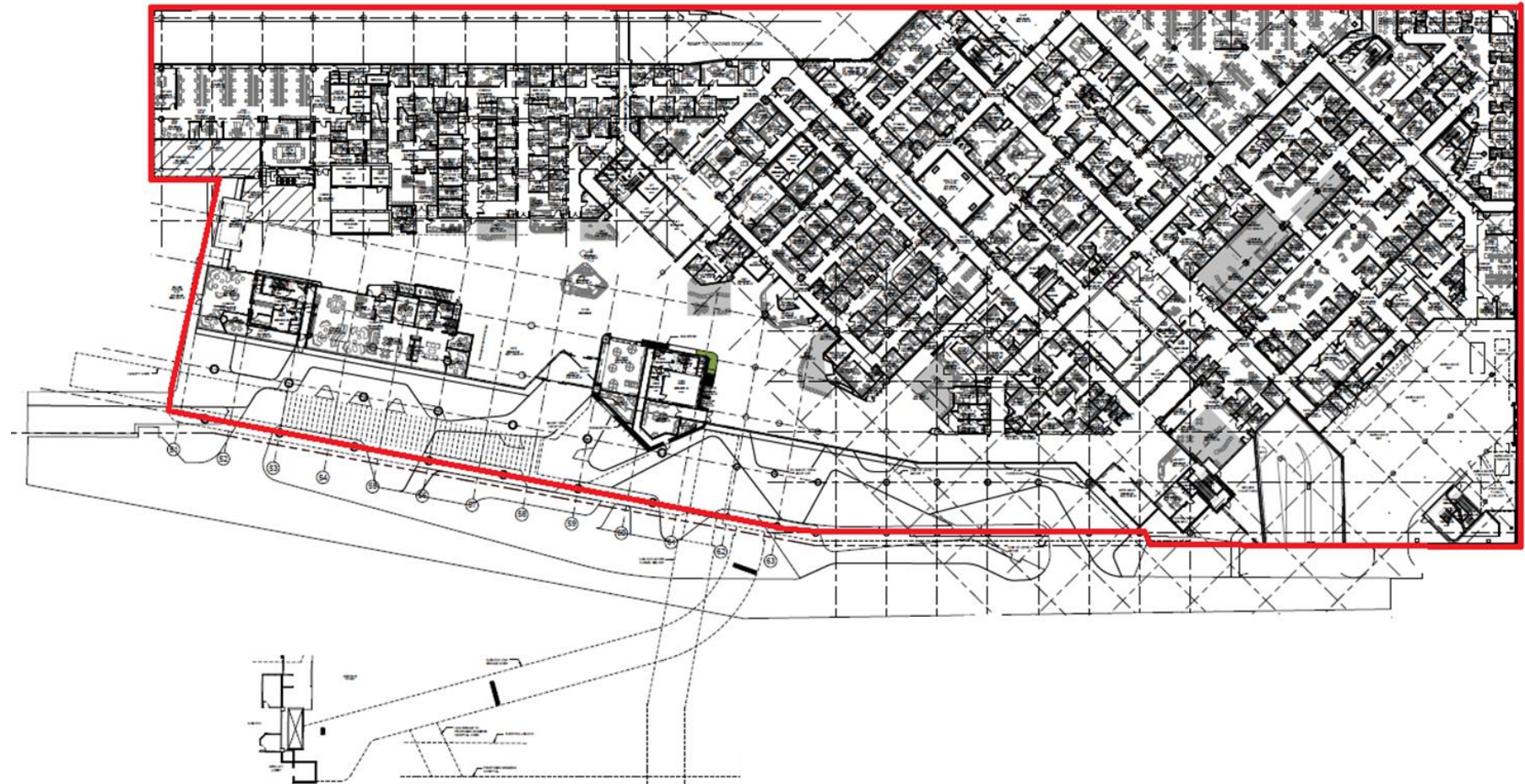
Advanced Paediatric Life Support (APLS). Paediatric basic life support [Internet]. Melbourne (VIC): APLS; 2017 [cited 2021 March 23]. Available from: <https://www.apls.org.au/algorithm-paediatric-basic-life-support>

Appendix 2: ARC BLS Algorithm



Australian Resuscitation Council (ARC). Flowcharts [Internet]. Melbourne (VIC): ARC; 2016 [cited 2021 March 23]. Available from: <https://resus.org.au/guidelines/flowcharts-3>

Appendix 3: Identification of boundary at PCH for Code Blue response (Ground Floor)



Appendix 4: Responder Roles in Resuscitation

These roles are outlined further in Action Cards within the **Emergency Procedures Manual: Code Blue**.

First Responder

1. Check for **danger** to self, patient or other people.
2. Check for **response** from the patient / person.
3. If unresponsive **send / call for help**
4. Activate the local emergency assistance button / ask someone to dial '**55**' - **Code Blue**.
State:
 - Hospital: '**PCH**'
 - Code: '**Code Blue**'
 - Location: 'level, department / area'
 - Your name and designation.
5. Do not leave patient unless there is no help arriving.
6. Commence basic life support.
7. Note the time of patient collapse.

Second Responder (or First Responder if no response)

1. Dial '**55**' and state:
 - Hospital: '**PCH**'
 - Code: '**Code Blue**'
 - Location: 'level, department / area'
 - Name and designation.
2. Collect resuscitation trolley and return to location of the patient.
3. Select appropriate size self-inflating bag-valve resuscitator, connect to oxygen at a flow of 10-15L/min; give to first responder to continue ventilation if required.
4. Place a cardiac board underneath the patient and perform external cardiac compressions if required until further help arrives.

Note: Self-inflating bag-valve mask:

- There are two types of self-inflating bag-valve resuscitators in use at PCH - Laerdal (non-disposable) and Mayo Healthcare (disposable) which both come in three sizes: Neonatal, Paediatric and Adult.
- Ensure you are familiar with the type of bag available in your area (refer to Resuscitation Equipment and Trolley Checking procedures and online learning packages).

Third Responder

1. Delegate a staff member to direct resuscitation team to appropriate area.
2. Commence drawing up and labelling:
 - 10mL Adrenaline **1:10,000** - dose is 0.1mL/ kg (IV/IO).
 - 10mL 0.9% Sodium Chloride (draw up several flushes).
3. Prepare for IV (or IO) insertion and administration of fluid bolus of 0.9% Sodium Chloride.
4. Commence (or delegate) **Code Blue** events recording; include drug administration, observations (minimum every 2 minutes), interventions and patient assessment.
5. At the end of the **Code Blue** or MET Review ensure that the original document is placed in the patient medical record and a copy of the record sent to the PCC Director

Other Staff

1. Remove the head of the bed and any excess furniture from the immediate area to facilitate access to the patient.
2. Ensure privacy and support is provided for family members who may be present. Where available a support person is to be allocated to stay with the family during this time and is to provide frequent and accurate updates using plain language.
3. Only those staff actively engaged in the resuscitation should be present.
4. Following the arrival of the PCC resuscitation trolley, consider the removal of the ward resuscitation trolley from the immediate area. Ward staff should continue to assist in the resuscitation as directed by the resuscitation leader.
5. Following medical emergency:
 - assist transfer of patient as required
 - complete documentation
 - save to the patient medical record and send copy to PCC Director
 - facilitate debrief as required
 - restock resuscitation trolley

Appendix 5: Hospital in the Home (HiTH) Early Warning Score Escalation Pathway

HiTH Early Warning Score Escalation Pathway	
Score	Clinical response
	• Remain vigilant • Complete a full set of observations when indicated • Handover to nurse in charge if concerned • A plan must be documented • Reassess EWS after interventions • Consider transfer to higher care • Responders to ensure patient team are aware of deterioration
1-3	Senior Nursing Review • Handover HITH coordinator • Consider a medical phone review • Consider the need to increase frequency of observations • Reassess EWS before leaving home visit • Educate parents/carers regarding signs of deterioration and provide them with a plan for review • Document plan in notes
4-5	Timely Medical Review • Arrange a medical review in hospital or via Telehealth • Handover to Hospital Clinical Manager (HCM) and ED consultant • Stay with the patient until transfer to hospital • Handover to HITH coordinator
6-7	Urgent Treating Team Review • Arrange ambulance transfer to hospital and provide handover to Emergency Departments Consultant. • Stay with the patient until transfer, continue to record a full set of observations every 15 minutes • Handover to treating medical team, HITH coordinator and HCM
8+	Rapid Response Review/ Ambulance • Initiate a medical emergency 000 call ask for urgent ambulance assistance • Assess the patient and initiate appropriate clinical care e.g., commence basic life support • Handover to medical team, HCM and HITH coordinator and Emergency Department Consultant
E	Immediate Medical Emergency • Initiate BLS as required
Emergency Call for any of the following: • Airway threat • Cardiac or respiratory arrest • Apnoea or cyanosis • Seizure/prolonged convulsion • Major bleeding • Severe respiratory distress • Any observation in the purple • You are worried about the patient	

Appendix 6: Hospital in the Home (HiTH) Paediatric Sepsis Recognition Escalation Pathway.

HITH Paediatric Sepsis Recognition Escalation Pathway	
Use if suspected infection OR abnormal temperature ($<36^{\circ}\text{C}$ or $\geq 38^{\circ}\text{C}$) Consider sepsis, bacterial infection and need for antibiotics. If sepsis recognition prompt not triggered respond as per early warning score.	
High risk patients - have a lower threshold for requesting medical review if: • Infants less than 3 months • Immunosuppression, chemotherapy, long-term steroids or asplenia • Invasive devices • Recent surgery, burn or wound • Unimmunised/incomplete immunisation • Rural, remote or low socioeconomic status • Re-presentation or delayed presentation	
Sepsis recognition prompt	Clinical response
EWS 6-7 OR any of the following • Mottled, CRT ≥ 3 or cold peripheries • Non-blanching rash • Drowsy or confused • Unexplained pain • Family and/or clinician concern is continuing or increasing	Urgent Medical Review • Arrange a medical review in hospital or via Telehealth • State sepsis review required • Arrange ambulance transfer to hospital and stay with the patient until transfer to hospital • Handover to Emergency Department Consultant, HITH coordinator, medical team and HCM • Continue to record a full set of observations every 15 minutes
EWS 8+ OR any of the following • Any observation in red zone • AVPU score P	Rapid Response Review/ Ambulance • Initiate a medical emergency 000 call ask for urgent ambulance assistance • State sepsis review required • Assess the patient and initiate appropriate clinical care e.g., commence basic life support • Handover to medical team, HCM and HITH coordinator and Emergency Department Consultant • Ask ambulance to assess lactate and BSL • Lactate >4 mmol/L • BSL <3 mmol/L